

METROPOLIS GENERAL HOSPITAL

Outpatient Urgent Care Clinic

CLINICAL ENCOUNTER SUMMARY

Patient Name:	Sarah Jenkins	Date of Visit:	May 22, 2026
Date of Birth:	March 14, 1991	Encounter ID:	MGH-2026-99214
Gender / Age:	Female / 35	Physician:	Dr. Marc Edwards, MD

1. CHIEF COMPLAINT & HISTORY

The patient is a 35-year-old female presenting with a 4-day history of productive cough, mild subjective fever, and generalized malaise. She notes the sputum is thick and yellowish. She reports associated right-sided pleuritic chest pain during deep inspiration. Denies shortness of breath at rest, hemoptysis, or recent travel. Past medical history is notable for mild intermittent asthma, managed with an albuterol inhaler as needed.

2. VITAL SIGNS

Blood Pressure 118/76 mmHg	Heart Rate 88 bpm	Temp (Oral) 38.2 °C	Resp. Rate 18 bpm	SpO2 96% (Air)
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3. DIAGNOSTIC FINDINGS

Physical Exam: Decreased breath sounds and mild crackles noted in the right lower lung field. No wheezing present.

Lab / Diagnostic Test	Result	Ref. Range	Status
White Blood Cell (WBC) Count	13.4 x10 ³ / μ L	4.5–11.0	High
Chest X-Ray (Single View)	Right lower lobe infiltrate	Clear lungs	Abnormal

4. ASSESSMENT & CLINICAL IMPRESSION

- Community-Acquired Pneumonia (CAP):** Right lower lobe, bacterial origin suspected given acute leukocytosis and localized infiltrate.
- Acute Asthma Exacerbation:** Currently quiescent, secondary to underlying respiratory infection.

5. PLAN OF CARE

- Amoxicillin-Clavulanate (Augmentin) 875/125 mg:** 1 tablet PO every 12 hours for 7 days.
- Albuterol HFA Inhaler:** 2 puffs every 4–6 hours PRN for cough or wheezing.
- Supportive Care:** Hydration, rest, and Acetaminophen 500 mg PO every 6 hours PRN for fever.
- Precautions:** Return immediately for worsening dyspnea, persistent high fevers, or inability to tolerate oral fluids. Follow up with primary care physician in 7–10 days.

Marc Edwards, MD

Electronically Signed / National Provider Identifier: 188274012